INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Vinorelbine and cisplatin · 28-day cycle

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00343
Version 7

A cycle lasts 28 days (4 weeks). Vinorelbine is given on Days 1, 8, 15, 22. Cisplatin is given on Days 1 and 8. Four cycles are planned, with review before doses. Delays or omitted doses may be needed for safety.

Treatment stages

01 **Chemotherapy**
Four 28-day cycles

02 **Review after chemotherapy**
Next plan agreed

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Vinorelbine and cisplatin · 28-day cycle · National Cancer Control Programme (NCCP) 00343

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your oncology team immediately for fever or sudden illness, shaking chills, breathing difficulty, chest pain, uncontrolled vomiting, very little urine, severe abdominal pain or constipation with vomiting. Tell the nurse immediately about burning or pain at the infusion site. Do not wait until the next visit.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

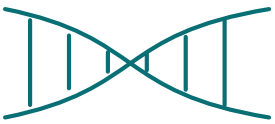
Three medicines. Different jobs.

After surgery, treatment aims to lower the chance of cancer returning. For advanced or recurrent lung cancer, it aims to control disease and symptoms. Your team should mark which goal applies to you; benefit is not guaranteed.

VINCA ALKALOID CHEMOTHERAPY

Vinorelbine

intravenous (IV) infusion



Disrupts microtubules needed for cell division

Vinorelbine disrupts the cell structures that help divide chromosomes. This can slow cancer growth and also affect blood cells, nerves and the bowel.

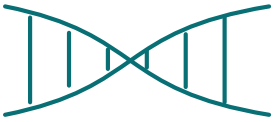
Binds cell-division structures → Microtubules cannot function normally → Cell division is disrupted

What to watch for

Report fever, constipation, numbness or pain at the infusion site promptly.

PLATINUM CHEMOTHERAPY

Cisplatin



Damages deoxyribonucleic acid (the cell's genetic material) (DNA) through platinum links

Cisplatin forms links in DNA that interfere with copying and repair. This helps stop cancer cells dividing, but healthy tissues may also be affected. Kidney protection with fluids and monitoring is central to cisplatin treatment.

Platinum reaches cells → DNA links form → Cell division disrupted

What to watch for

Report ringing in the ears, hearing changes, numbness or loss of function. Tell the team if you cannot drink, are passing less urine or become swollen or breathless during fluids.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment and its aim

Why two medicines?

The medicines interfere with cancer-cell division in different ways. They can also affect healthy cells.

Choosing treatment

Discuss likely benefit, side effects, alternatives and what matters most to you.

Evidence and uncertainty

Trials support cisplatin-based chemotherapy after surgery for selected lung cancers. The benefit depends on the stage and treatment goal; evidence after surgery cannot be used as a prognosis for advanced cancer. JBR.10 studied resected stage IB/II disease. Its schedule matches the 28-day split cisplatin regimen more closely. No individual survival estimate or metastatic benefit percentage is provided here.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Vinorelbine and cisplatin · 28-day cycle · NCCP 00343

Treatment → review → next steps

Treatment stages

01 **Chemotherapy**
Four 28-day cycles

02 **Review after chemotherapy**
Next plan agreed

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

FOUR 28-DAY CYCLES

Chemotherapy

1 2 3 4

A cycle lasts 28 days (4 weeks). Vinorelbine is given on Days 1, 8, 15, 22. Cisplatin is given on Days 1 and 8. Four cycles are planned, with review before doses. Delays or omitted doses may be needed for safety.

NEXT PLAN AGREED

Review after chemotherapy

Review

Scans and follow-up depend on the treatment goal. Surgery or radiotherapy is not automatically part of this regimen; ask where any separately planned treatment fits.

Your team confirms each stage and discusses any changes with you.

BASELINE → TREATMENT → FOLLOW-UP

Your treatment and checks

Relative stages, not equal time intervals. There is no routine heart-scan schedule in this regimen.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00343 v7 · official regimen

THE WHOLE TREATMENT JOURNEY

Vinorelbine and cisplatin · 28-day cycle: treatment and checks

Postoperative adjuvant or advanced/recurrent non-small-cell lung cancer (NSCLC): confirm your treatment goal

A cycle lasts 28 days (4 weeks). Vinorelbine is given on Days 1, 8, 15, 22. Cisplatin is given on Days 1 and 8. Four cycles are planned, with review before doses. Delays or omitted doses may be needed for safety.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Preparation	Cycle 1	Cycle 2	Cycle 3	Cycle 4	After treatment
Treatment NCCP 00343	Confirm goal and consent	28-day schedule	Repeat if appropriate	Review benefit and effects	Final planned cycle	Agree next plan
Blood tests NCCP 00343	Blood count, kidneys, liver	Blood count weekly	Kidneys/ liver before cycle	Continue weekly blood count	Review recovery	Follow abnormalities
Hearing / nerves / bowels NCCP 00343	Record baseline symptoms	Ask before each dose	Report any change	Adjust if needed	Document lasting effects	Follow symptoms until resolved
Scans / other treatment NCCP 00343	Stage and multidisciplinary team (MDT) plan	As individually planned	No universal scan date	Consultant review	End-of-course review	Surveillance or next treatment

What each check is for

1 Preparation

Before cycle 1

Confirm treatment goal

Ask / act: Document adjuvant versus advanced goal and contact details.

2

Weekly review

Before weekly vinorelbine

Counts and symptoms can change

Ask / **act:** Review source-specific day/count/history actions.

3 **Cisplatin visits**

Day 1 and8

Protect kidneys and manage sickness

Ask / **act:** Confirm hydration, electrolytes, interactions and prescription.

4 **Course completion**

After four planned cycles

Agree follow-up

Ask / **act:** Review recovery, disease assessment and next plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the visit

Confirm weekly blood tests, transport, medicines and your daytime/out-of-hours oncology numbers.

STEP 2

Time in the unit

Vinorelbine is infused over15 minutes, followed by a saline flush. Cisplatin is infused over about one hour on its treatment days, with fluids before and afterwards. Checks, preparation and support medicines make the appointment longer than the infusion itself.

STEP 3

IV access

Suitable IV access is required. No routine take-home pump. Tell the nurse promptly about pain or burning.

STEP 4

Home support

Follow your prescribed anti-sickness and constipation-prevention plan. Cisplatin days need kidney-protecting fluids and electrolyte checks. Do not repeat the full cisplatin anti-sickness course on a vinorelbine-only day unless prescribed.

STEP 5

After the course

Agree follow-up, scans and how to report persistent problems.

Your visits

Before each dose

Blood count every week; kidney and liver tests before each cycle. Report hearing changes, tingling, bowel problems and poor fluid intake before every dose. Extra tests and scans follow your individual plan.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

Vinorelbine and cisplatin · 28-day cycle · NCCP 00343

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Vinorelbine visits	1 28 **Days 1–22** Days 1, 8, 15, 22 only; not a continuous infusion.
Watch for symptoms every day	1 28 **Days 1–28** Blood counts and symptoms vary; the calendar cannot predict a safe day.

A schedule, not a prediction: Symptoms may occur on any day. Treatment depends on blood results and how you are feeling.

Your overall plan: Four cycles take approximately 16 weeks if there are no delays. Confirm the intended goal, number of cycles and any separate local treatment.

Why these bands stay broad: Calendar shows planned visits only. No patient dates are required. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

There are no guaranteed "safe days". Call your oncology team immediately for fever or sudden illness, shaking chills, breathing difficulty, chest pain, uncontrolled vomiting, very little urine, severe abdominal pain or constipation with vomiting. Tell the nurse immediately about burning or pain at the infusion site. Do not wait until the next visit.

Between-treatment notes

Day 1: Day 1: Vinorelbine and cisplatin

Treatment after clinical and blood-test review. Tell the team about new symptoms before treatment.

Day 2: Day 2: At home

Effects vary. Keep taking only the support medicines prescribed for today. Use your contact plan if unwell; do not wait for a treatment day.

Day 3: Day 3: At home
Day 4: Day 4: At home
Day 5: Day 5: At home
Day 6: Day 6: At home
Day 7: Day 7: At home
Day 8: Day 8: Vinorelbine and cisplatin
Day 9: Day 9: At home
Day 10: Day 10: At home
Day 11: Day 11: At home
Day 12: Day 12: At home
Day 13: Day 13: At home
Day 14: Day 14: At home
Day 15: Day 15: Vinorelbine
Day 16: Day 16: At home
Day 17: Day 17: At home
Day 18: Day 18: At home
Day 19: Day 19: At home
Day 20: Day 20: At home
Day 21: Day 21: At home
Day 22: Day 22: Vinorelbine
Day 23: Day 23: At home
Day 24: Day 24: At home
Day 25: Day 25: At home
Day 26: Day 26: At home
Day 27: Day 27: At home
Day 28: Day 28: At home

Support around the medicines

1 **Bowel care**

Take the prescribed laxative plan. Tell the team if your bowels slow down; severe pain, swelling or vomiting needs urgent advice.

2 **Protect kidneys, hearing and nerves**

Follow your individual drinking advice and report reduced urine, ringing ears, hearing changes or tingling. Ask before taking new medicines or supplements.

Side effects and when to call

Infection

Fever, shaking chills or sudden illness can be serious even between visits.

Bowel symptoms

Report constipation early; severe pain or vomiting needs urgent advice.

Hearing and nerves

Tell the team promptly about hearing changes, ringing ears, tingling or weakness.

Everyday life

Adjust activity and support to how you feel.

Food and fluids

Ask for help with poor appetite or sickness. Follow individual hydration advice, including any fluid restriction.



Medicines and pregnancy

Check medicines and supplements with the team. Discuss fertility, contraception and breastfeeding before treatment.

After chemotherapy

Your next plan depends on why treatment was given.

Other treatments

Surgery and radiotherapy require separate plans; neither is automatically added by this schedule.

Ongoing effects

Report persistent hearing or nerve changes and attend kidney/blood checks as arranged.

Contacts and questions

Keep your systemic anticancer therapy (SACT) card and contact details available.

Check your understanding

Ask the team to confirm your treatment name, next treatment day and what to do if you feel unwell.

Questions worth asking

What if a weekly dose is missed?

The team decides whether to omit, delay or change a dose. Do not add a missed dose yourself; the two vinorelbine/cisplatin schedules have different rules.

Will I also have radiotherapy?

It is not built into this chemotherapy schedule. Ask your team to explain any separately planned radiotherapy or other treatment.

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cisplatin-containing day: 1/8

Active medicines: vinorelbine + cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) on Day 1, then daily on Days 2–3 Aprepitant inhibits CYP3A4 and can increase vinca toxicity; prescriber/ pharmacy must review the combination and monitor.
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	General high-risk NCIS V8 row Reconcile the antiemetic course and interactions into one prescription; this is a reference course requiring local confirmation.
Olanzapine	5 mg orally once daily	Day 1 and Days 2–4 General high-risk NCIS reference; review sedation, falls, metabolic risk and interaction with metoclopramide. Consultant may individualise.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability. Co-prescribing with olanzapine increases extrapyramidal risk; assess neurological contraindications and interactions.

Additional phase-specific support

- Follow your prescribed anti-sickness and constipation-prevention plan. Cisplatin days need kidney-protecting fluids and electrolyte checks. Do not repeat the full cisplatin anti-sickness course on a vinorelbine-only day unless prescribed.
- Cisplatin pre/post hydration and electrolyte prescription per local policy; monitor fluid balance.
- Prophylactic bowel plan; assess CYP3A interactions, including aprepitant.
- This reference is not a prescription; doses and duration require local oncology/pharmacy confirmation.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Vinorelbine-only day: 15/22

Active medicines: vinorelbine

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis for minimal risk; individualise rescue medicines.
- Continue the prescribed constipation plan. Do not restart the full cisplatin antiemetic course automatically.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Your anatomical side-effect map

Vinorelbine and cisplatin · 28-day cycle

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hearing and nerves

Cisplatin / vinorelbine

Report ringing ears, hearing change, numbness or weakness; effects can persist.

3 Kidneys / fluids

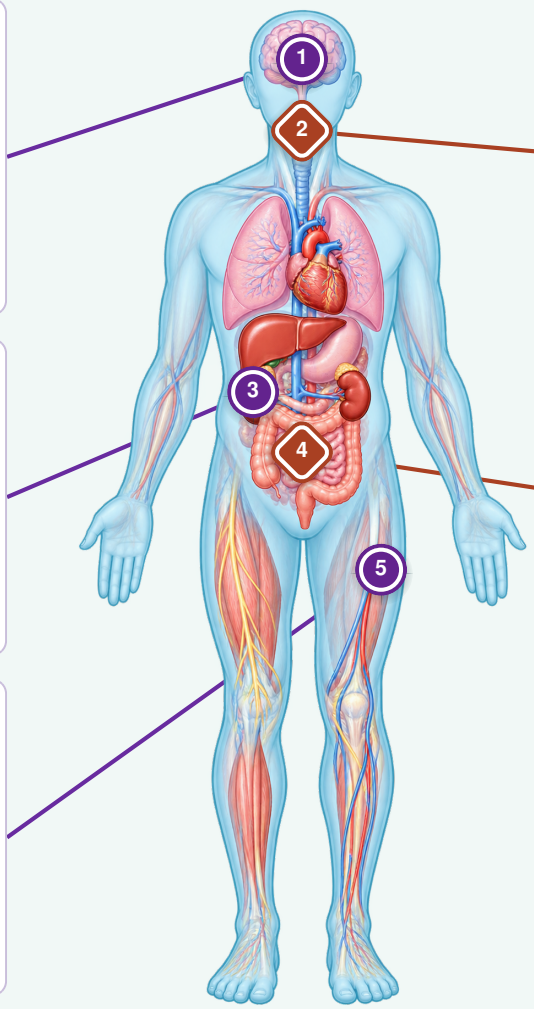
Cisplatin

Fluids protect kidneys; call for very little urine or inability to drink.

5 Blood cells / infusion site

Both medicines

Fever or sudden illness needs immediate advice. Tell the nurse about infusion pain.



2 Mouth

Both medicines

Report mouth sores that make eating or drinking difficult.

4 Bowels / sickness

Both medicines

Constipation and nausea need early treatment; severe pain or vomiting needs urgent advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

SACTCHECK · CHAPTER 04

Sources and further information

Vinorelbine and cisplatin · 28-day cycle · National Cancer Control Programme (NCCP) 00343

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00343 v7 · official regimen**
   https://healthservice.hse.ie/documents/6916/343_V7_Vin25Cis50.pdf
2. **NCCP NCIS antiemetics V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
3. **JBR.10 long-term randomized adjuvant trial**
   https://pubmed.ncbi.nlm.nih.gov/19933915/
4. **NCIC JBR.10 study schedule**
   https://www.ctg.queensu.ca/public/asco/br10-asco-2004.pdf